

Patient ID : PA02

CT Scan of Chest and Abdomen

Report:

Plain (oral contrast only) axial scans were taken in the region of interest. Intravenous contrast-enhanced axial scans were taken from the thoracic inlet to the pubic symphysis. Study reveals:

Trachea & bronchi: Trachea is normal in diameter, shape, position & bifurcation. No evidence of any narrowing or intraluminal mass lesion. Right and left main bronchi are normal in size & diameter.

Lungs & Pleura: Mild bilateral pleural effusion is seen with consolidation basal segment both lower lobes and patchy illdefined areas of ground glass attenuation in posterior segment bilateral upper lobes.

Rest of lung parenchyma is normal in attenuation. No evidence of interstitial thickening. No lung mass or nodule is seen. No evidence of pleural thickening or calcification.

Mediastinum: Heart & major mediastinal vessels are normal. No abnormal mediastinal lymphadenopathy is seen. No mass lesion is seen. No evidence of pericardial effusion.

Chest wall & axilla: Chest wall musculature is normal. Bony thorax is normal. Bilateral axillae are normal. No evidence of axillary lymphadenopathy

Liver: - Is normal in size and shows diffuse fatty changes. Liver enhancement is normal. No evidence of intrahepatic biliary radical dilatation. No focal space-occupying lesion is seen.

Gall bladder: - Is normal in diameter and wall thickness. No hyperdense intraluminal calculus is seen.

CBD: - is normal in diameter, with normal walls. No hyperdense intraluminal calculus is seen. **Portal vein is normal in calibre. SP axis is patent.**

Pancreas: - Is normal in size, attenuation and enhancement. Pancreatic duct is not dilated. No evidence of pancreatic calcification. No evidence of any peripancreatic inflammation or fluid collection.

Spleen: - Is normal in size, attenuation and enhancement. No focal space-occupying lesion is seen.

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Kidneys: - Both kidneys are normal in size, position and attenuation. Kidney enhancement is normal and corticomedullary differentiation is maintained. No evidence of hydronephrosis, no calculi seen.

Bilateral **adrenal glands** are normal in size and attenuation.

Urinary Bladder: - Is partially distended, normal in position. Bladder walls are not thickened, no mass lesion is seen. Foleys catheter bulb is seen in situ. No intraluminal calculus is seen.

Uterus: - Is normal in size and shows presence of a small (22 mm) intramural exophytic hypodense mildly enhancing lesion in anterior wall body of uterus. Endometrial measures~6 mm in thickness.

Bilateral adnexal region is seen normally.

Aorta and IVC are normal. No evidence of retroperitoneal or mesenteric lymphadenopathy. No evidence of ascites.

No abnormal bowel wall thickening or mass lesion is seen. No abnormally dilated bowel loops is seen. Ileocecal junction and cecum is seen normally.

Impression: Mild bilateral pleural effusion with consolidation basal segment both lower lobes and patchy illdefined areas of ground glass attenuation in posterior segment bilateral upper lobes likely infective.

Small intramural exophytic hypodense mildly enhancing lesion in anterior wall body of uterus likely leiomyoma.

Diffuse fatty changes in liver.

Advise: Clinical Correlation.